

Parent's Decision Guide

A step-by-step guide to help you decide what to do after your child bumps or injures their head.



Call 1011 (Police) / 10177 (Ambulance) / 112 (mobile) immediately if your child is unconscious, not breathing, or having a seizure.

Response Levels

Outcomes in this guide use four colour-coded levels.



Emergency (10177 / 112)
Life-threatening – call now



Emergency Dept
Go to hospital urgently



See a Paediatrician
or after-hours clinic today



Home Care
Monitor at home

Decision Tree

Tap each question to reveal guidance. Work through them in order.

1 Is your child unconscious, unresponsive, or not breathing normally?

YES



Call 10177 (Ambulance) or 112 (mobile)

Do not move the child unless they are in immediate danger. Stay on the line with the operator.

NO

Good — continue to Question 2.

2 Is your child having a seizure (fit) or has had one since the injury?

YES	 Call 10177 (Ambulance) or 112 (mobile) Lay the child on their side (recovery position). Time the seizure. Don't put anything in their mouth.
NO	Continue to Question 3.

3 **Is there blood or clear fluid coming from the ears or nose (not from a cut)?**

YES	 Call 10177 (Ambulance) or 112 (mobile) This may indicate a skull base fracture. Do not plug the ear or nose.
NO	Continue to Question 4.

4 **Does your child have a very large, soft, boggy lump on the head (not a firm, regular bump)?**

YES	 Go to Emergency Department Especially concerning in infants under 12 months. A soft swelling on the scalp may indicate bleeding under the skull.
NO	A firm egg-shaped bump (haematoma) is very common and usually harmless. Continue to Question 5.

5 **Did your child lose consciousness — even briefly (went limp or eyes rolled)?**

YES	 Go to Emergency Department Even a few seconds of unconsciousness requires medical assessment for concussion or more serious injury.
NO	Continue to Question 6.

6 Is your child very difficult to wake, extremely drowsy, or acting very confused / "not themselves"?

YES

 **Go to Emergency Department**

Note: some sleepiness right after the fall is normal, but your child should be rousable and recognise you.

NO

Continue to Question 7.

7 Has your child vomited 3 or more times since the injury?

YES

 **Go to Emergency Department**

Vomiting once or twice is common after a head knock. Repeated vomiting (≥ 3 times) needs urgent review.

NO

Continue to Question 8.

8 Does your child have a severe headache that is not improving, or a stiff neck?

YES

 **Go to Emergency Department**

Worsening or persistent severe headache after head injury needs urgent review.

NO

Continue to Question 9.

9 Is your child under 12 months old AND had a fall of any height onto a hard surface?

YES	See a Paediatrician Today Infants' skulls are more vulnerable. Even if they seem fine, a same-day review by a doctor is recommended.
NO	Continue to Question 10.

10

Was the fall from a height greater than your child's own height, or onto a very hard surface (concrete, tiles)?

YES	See a Paediatrician Today High-energy impacts warrant same-day review even without obvious symptoms.
NO	Continue to Question 11.

11

Does your child have a cut or wound on the head that won't stop bleeding after 10 minutes of firm pressure, or is gaping open?

YES	See a Paediatrician / Emergency Dept The wound may need cleaning, glue, steri-strips, or stitches. Apply firm pressure with a clean cloth while travelling.
NO	Continue to Question 12.

12

Is your child generally back to normal — crying briefly then settling, playing, feeding, making eye contact?

YES	 Home Care & Monitor This is reassuring. See the home monitoring advice below. You should still check on your child every few hours for the next 24 hours.
NO	 See a Paediatrician Today If you're unsure or something doesn't feel right, always trust your instincts and seek medical advice.

Warning Signs — Return to ED Immediately If:

Even if your child was initially fine, go to hospital if any of these develop in the next 48 hours.

 Cannot be woken or extremely drowsy	 Repeated vomiting (≥ 3 times)	 Seizure or jerking movements
 Slurred speech or confusion	 Unsteady walking or loss of balance	 One pupil larger than the other
 Headache that keeps getting worse	 Unusual behaviour or won't stop crying	 Weakness in arms or legs
 Sleeping much more than normal		

Home Care Advice

For minor head bumps where your child is well and back to normal.

Ice pack for the bump

Wrap ice or a frozen pack in a cloth. Apply for 10–15 minutes to reduce swelling. Never put ice directly on skin.

Pain relief

Paracetamol (e.g. Panadol Children's) at the correct dose for your child's weight is appropriate for pain or headache. Avoid ibuprofen in the first few hours as it may mask symptoms. Avoid aspirin in children.

Rest for the remainder of the day

Quiet play is fine. Avoid vigorous activity, contact sports, or screen time for at least 24 hours after a significant knock.

Sleep — it is OK to let them sleep

It is safe to let your child sleep. You do not need to wake them every hour. Do check on them and make sure they are rousable and normal colour.

Monitor for 24–48 hours

Check on your child frequently. Review the warning signs above. If anything worries you, see a paediatrician or go to emergency.

School and sport

Keep them home from sport until symptom-free. Return to school gradually. Speak to your paediatrician if symptoms persist beyond a few days.

Prepared by your paediatrician for guidance only. This guide does not replace professional medical assessment. If you are ever unsure about your child's condition, seek medical advice promptly. Trust your parental instincts — if something doesn't seem right, get it checked.

Emergency: **10177** (Ambulance) · **10111** (Police) · **112** (any mobile) · Private ambulance: **ER24: 084 124** · **Netcare 911: 082 911**